

apex	apex laboratories private limited Plant-I	Date : 16.08.2023
Title:	Zincofer Capsules (Leaflet) Malaysia	AWC : B3PMS311/00

Leaflet Size: W:160 x L:190 mm

Front

Back

For the use of Registered Medical Practitioner or a Hospital or a Laboratory only.

Zincofer Capsules

(Haematinic Capsules)

DESCRIPTION:
Scarlet / Scarlet coloured '0' size capsule containing brownish grey coloured granular powder.

COMPOSITION:
Each capsule contains:

Ferrous Fumarate	BP	350 mg
Providing Iron 115 mg		
Cyanocobalamin	BP	5 mcg
Folic Acid	BP	1 mg
Ascorbic Acid	BP	75 mg
Pyridoxine Hydrochloride	BP	1.5 mg
Providing Pyridoxine 1.23 mg		
Zinc Sulfate Monohydrate	USP	55 mg
Providing Zinc 20 mg		
Sodium Benzoate (as preservative)	BP	0.3 mg

PHARMACODYNAMICS:
Zincofer is haematinic offering Zinc with Iron and other important haemopoietic factors. **Zincofer** initiates DNA, RNA synthesis and ensures proper foetus formation. **Zincofer** prevent foetus malformations due to Zinc deficiency in pregnancy. **Zincofer** corrects symptoms like perverted taste, loss of appetite and mental depression often observed during pregnancy. **Zincofer** corrects anaemia. There is evidence that a daily dose of 100mg of Elemental Iron in the Ferrous form is adequate to prevent development of Iron deficiency in expectant mothers. If a mild Iron deficiency is present when **Zincofer** administration is started, this will be corrected by increased absorption of Iron. The daily folate requirement rises steeply during the final trimester of pregnancy, and evidence of maternal depletion may be found. To ensure normal tissue folate levels in the mother after delivery a daily supplement of about 300 micrograms is required during the second and third trimester of pregnancy. This does not obscure the blood picture of Addisonian pernicious anaemia.

PHARMACOKINETICS:
Iron is absorbed chiefly in the duodenum and jejunum, absorption being aided by the acid secretion of the stomach and being more readily effected when the Iron is in the Ferrous state. Folic Acid is absorbed mainly from the proximal part of the small intestine. Folate polyglutamates are considered to be de-conjugated to monoglutamates during absorption. Folic Acid rapidly appears in the blood, where it is extensively bound to plasma proteins. The amounts of Folic Acid absorbed from normal diets are rapidly distributed in body tissues and about 4 to 5 micrograms is excreted in the urine daily. When larger amounts are absorbed, a high proportion is metabolised in the liver to other active forms of folate and a proportion is stored as reduced and methylated folate. Larger amounts of folate are rapidly excreted in the urine.

INDICATIONS:
Zincofer is indicated in Iron deficiency anaemia, nutritional deficiency anaemia and anaemia associated with pregnancy, worm infestation etc. **Zincofer** are indicated during the second and third trimester of pregnancy for prophylaxis against Iron deficiency and megaloblastic anaemia of pregnancy.

SOURCE OF CAPSULE:
Bovine

RECOMMENDED DOSE:
One capsule a day after meals or as prescribed by the Physician.
Adults: It is usual to begin therapy with **Zincofer** about the thirteenth week of pregnancy (see precautions) either as routine prophylaxis or selectively if the haemoglobin concentration is less than 11g/100 ml (less than 75% normal). One capsule should be taken daily by mouth.
Children: Not applicable.

ROUTE OF ADMINISTRATION:
Oral route

CONTRAINDICATIONS:
Known hypersensitivity to the product, Vitamin B12 deficiency, paroxysmal nocturnal haemoglobinuria, haemosiderosis, haemochromatosis, active peptic ulcer, repeated blood transfusion, regional enteritis and ulcerative colitis. **Zincofer** must not be used in the treatment of anaemias other than those due to Iron deficiency.

WARNING AND PRECAUTIONS:
The label will state "Important warning: Contains Iron. Keep out of reach and sight of children, as overdose may be fatal". This will appear on the front of the pack within a rectangle in which there is no other information. Some post-gastroctomy patients show poor absorption of Iron. Care is needed when treating iron deficiency anaemia in patients with treated or controlled peptic ulceration. Caution should be exercised when administering Folic Acid to patients who may have folate dependent tumours. Since anaemia due to combined Iron and Vitamin B12 or folate deficiencies may be microcytic in type, patients with microcytic anaemia resistant to therapy with iron alone should be screened for Vitamin B12 or folate deficiency. **Zincofer** should be kept out of reach & sight of children.

DRUG INTERACTIONS:
Concurrent administration with antacids. Iron reduces the absorption of penicillamine. Iron compounds impair the bioavailability of fluoroquinolones, levodopa, carbidopa, thyroxine and bisphosphonates. Absorption of both Iron and antibiotic may be reduced if **Zincofer** is given with tetracycline. Concurrent administration of antacids may reduce absorption of Iron. Co-trimoxazole, chloramphenicol, sulphasalazine, aminopterin, methotrexate, pyrimethamine or sulphonamides may interfere with folate metabolism. Serum levels of anticonvulsant drugs may be reduced by administration of folate. Oral chloramphenicol delays plasma iron clearance, incorporation of Iron into red blood cells and interferes with erythropoiesis. Some inhibition of Iron absorption may occur if it is taken with cholestyramine, trientine, tea, eggs or milk. Administration of oral Iron may increase blood pressure in patients receiving methyl dopa. Coffee may be a factor in reducing Iron bioavailability. Neomycin may alter the absorption of Iron.

PREGNANCY AND LACTATION:
Since **Zincofer** are haematinic, safely given during pregnancy and lactation. Administration of **Zincofer** during the first trimester of pregnancy may be undesirable. A minority of pregnant women are not protected by physiological doses of Folic Acid. The development of anaemia despite prophylaxis with **Zincofer** calls for investigation.

SIDE EFFECTS:
Gastro-intestinal disorders have been reported including gastro-intestinal discomfort, anorexia, nausea, vomiting, constipation, diarrhoea. Darkening of the stools may occur. Rarely allergic reactions may occur.

SYMPTOMS AND TREATMENT OF OVERDOSE:
Acute overdose of oral Iron requires emergency treatment. In young children 200-250 mg/kg Ferrous Fumarate is considered to be extremely dangerous. Symptoms and signs of abdominal pain, vomiting and diarrhoea appear within 60 minutes. Cardiovascular collapse with coma may follow. Some improvement may occur after this phase which, in some patients, is followed by recovery. In others, after about 16 hours, deterioration may occur involving diffuse vascular congestion, pulmonary oedema, convulsions, anuria, hypothermia, severe shock, metabolic acidosis, coagulation abnormalities and hypoglycaemia. Vomiting should be induced immediately, followed as soon as possible by parenteral injection of desferrioxamine mesylate, and then gastric lavage. In the meantime, it is helpful to give milk and/or 5 % sodium bicarbonate solution by mouth. Dissolve 2g desferrioxamine mesylate in 2 to 3 ml of water for injections and give intramuscularly. A solution of 5g desferrioxamine in 50 to 100 ml of fluid may be left in the stomach. If desferrioxamine is not available, leave 300 ml of 1 % to 5 % sodium bicarbonate in the stomach. Fluid replacement is essential. Recovery may be complicated by long-term sequelae such as hepatic necrosis, pyloric stenosis or acute toxic encephalitis which may lead to CNS damage.

STORAGE CONDITION:
Store in a dry place below 30°C, and protect from light.

SHELF LIFE:
24 Months

PRESENTATION:
10 x 10 capsules in Blister pack.

apex
Manufactured by:
apex laboratories private limited
B-23 (Plant 1), SIDCO Pharmaceutical Complex,
Alathur - 603110, Kancheepuram District, India.

Revised on Date: 10 July 2023

Product Registration Holder:
Unimed Sdn Bhd
53, Jalan Tembaga SD 5/2B,
Bandar Sri Damansara 52200,
Kuala Lumpur, Malaysia.

B3PMS311/00

Dimension	4 Color/ Pantone	Type of board	Type of varnish	Embossing	Unvarnished Zone	GSM	Type of Fold
W:160 x L:190 mm	BLACK	MAPLITHO	—	—	—	60 Gsm	2 fold W: 40mm x L: 190mm
Initiated by		Reviewed by				Approved by	
Product Manager	Design	RA	R&D	Production	QC	Head - QA	
—	Nattananjanah	D-14	—	—	—	—	
Name :	Azharudeen	C.YESUDAS	ANBALAGAN	P.SANKAR	E.MOHANAKUMAR	SUNISHA V.A	
Date :	16/08/2023	16/08/2023	17/08/23	18/08/23	18/08/23	18/08/23	

Revision History		
S.No	From	To
1.	B3PMS088/00	B3PMS311/00
2.	Size: 110x170 mm	W160mm x L190mm
3.	B-23, SIDCO Pharmaceutical Complex, Alathur-603110, Tamil Nadu, INDIA	B-23 (Plant 1), SIDCO Pharmaceutical Complex, Alathur-603110, Kancheepuram District, India
4.	Revised on Date: 07 April 2021	Revised on Date: 10 July 2023